

HOSPITAL DISCHARGE SUMMARY

Riverside General Hospital — Department of Internal Medicine

REASON FOR ADMISSION

The patient was admitted from the emergency department for further evaluation and management of five days of progressively worsening shortness of breath, productive cough with yellow-green sputum, and subjective fevers. Admission was pursued given hypoxia noted on initial assessment and radiographic findings concerning for a lower respiratory tract infection.

HISTORY OF PRESENT ILLNESS

Symptoms began approximately one week prior to admission with mild nasal congestion and sore throat, initially attributed to a viral upper respiratory infection. Over the subsequent several days, the patient developed a productive cough, intermittent chills, and increasing dyspnea on exertion, eventually experiencing breathlessness with minimal activity such as walking across a room. Associated symptoms included pleuritic chest discomfort on deep inspiration and generalized fatigue. There was no associated hemoptysis, lower extremity swelling, or orthopnea. The patient denied recent travel, known sick contacts, or any similar episode within the past year, and reported no improvement despite over-the-counter cold remedies taken in the days prior to admission.

PAST MEDICAL HISTORY

The patient's known medical history is notable for well-controlled hypertension managed with a single oral antihypertensive agent, and mild intermittent asthma treated with an as-needed short-acting bronchodilator inhaler, with no prior hospitalizations for respiratory illness. There is a distant history of seasonal allergic rhinitis. The patient has no known history of chronic obstructive pulmonary disease, congestive heart failure, diabetes, or immunocompromising conditions. Vaccination history includes prior receipt of influenza and pneumococcal vaccination, though the timing of the most recent doses was uncertain at the time of admission.

SOCIAL AND FAMILY HISTORY

The patient reports never having smoked and denies any current or past recreational drug use, with only occasional social alcohol consumption. There is no reported occupational exposure to respiratory irritants, dusts, or fumes. Family history is notable for hypertension in a first-degree relative, but no known family history of chronic lung disease, early cardiac disease, or malignancy.

ALLERGIES

No known drug allergies were reported or documented during this admission.

HOSPITAL COURSE

On admission, the patient was noted to be mildly tachypneic with oxygen saturation in the low 90s on room air, which improved to the mid-to-high 90s with two liters of supplemental oxygen via nasal cannula. Initial chest imaging demonstrated a focal area of consolidation involving the right lower lobe, consistent with the working diagnosis of community-acquired pneumonia. Empiric intravenous antibiotic therapy was initiated promptly following admission, in accordance with standard institutional protocols for community-acquired pneumonia, and blood and sputum cultures were obtained prior to the first antibiotic dose.

Over the first forty-eight hours of admission, the patient's respiratory status gradually improved, with decreasing supplemental oxygen requirements and improved work of breathing on examination. Fevers, present intermittently during the first two hospital days, resolved by the morning of hospital day three, after which the patient remained afebrile for the remainder of the admission. Sputum culture results returned with growth of a common respiratory pathogen sensitive to the antibiotic regimen already in use, allowing the treatment team to continue the existing therapy without need for escalation or a change in antimicrobial coverage. The patient was transitioned from intravenous to oral antibiotic therapy on hospital day four, tolerating the medication well without gastrointestinal side effects.

Throughout the admission, the patient remained hemodynamically stable, with no episodes of hypotension, arrhythmia, or altered mental status. Ambulation and physical therapy were encouraged early in the admission to prevent deconditioning, and the patient was noted to tolerate increasing activity levels each day, progressing from bed-to-chair mobility to independent ambulation in the hallway without supplemental oxygen by hospital day five. Nutritional intake was adequate throughout the stay, and the patient's home antihypertensive medication was continued without interruption, with blood pressure remaining well controlled throughout the hospitalization. Respiratory therapy was involved briefly to reinforce inhaler technique and incentive spirometry, both of which the patient performed correctly and consistently by the second hospital day.

ASSESSMENT AND CLINICAL REASONING

The overall clinical picture on admission — subacute onset of productive cough, fever, pleuritic chest pain, and hypoxia in the setting of a new focal consolidation on chest radiograph — was most consistent with community-acquired bacterial pneumonia rather than a purely viral process, given the degree of hypoxia and the radiographic findings. Alternative diagnoses considered on admission included an asthma exacerbation with a superimposed infection, which was felt less likely as the primary driver given the focal nature of the consolidation and the degree of hypoxia, though the patient's underlying mild asthma was treated concurrently as a precaution. Heart failure was considered given the presenting dyspnea but was felt unlikely given the absence of lower extremity edema, a normal cardiac examination, and the focal rather than diffuse pattern of the radiographic findings.

The subsequent clinical trajectory, with steady improvement following initiation of antibiotic therapy and a sputum culture identifying a typical bacterial respiratory pathogen, was consistent with and supportive of the working diagnosis of community-acquired pneumonia.

PHYSICAL EXAMINATION FINDINGS (AT TIME OF DISCHARGE)

On the day of discharge, the patient appeared comfortable and in no acute distress, breathing room air without difficulty. Auscultation of the lungs revealed clear breath sounds bilaterally, with only trace residual crackles noted at the right lung base, a substantial improvement from the coarse crackles and diminished breath sounds documented on admission. Cardiovascular examination revealed a regular rate and rhythm without murmurs, gallops, or rubs. The abdomen was soft and non-tender, and no lower extremity edema was present. Skin was warm and well perfused without rash. The patient was alert, fully oriented, and ambulating steadily without assistance, with a normal, unlabored gait.

DIAGNOSTIC STUDIES SUMMARY

Chest radiography performed on admission demonstrated a focal right lower lobe consolidation consistent with pneumonia, without associated pleural effusion. A repeat chest radiograph obtained prior to discharge showed substantial interval improvement in the consolidation, with only mild residual opacity remaining. Laboratory studies obtained during the admission, including a complete blood count and basic metabolic panel, trended toward normalization over the course of the stay, and blood cultures remained without growth throughout the admission, arguing against bacteremia. Sputum culture, as noted above, identified an organism sensitive to the antibiotic regimen administered. No further imaging or invasive diagnostic studies were required during this admission.

CONDITION AT DISCHARGE

At the time of discharge, the patient was afebrile, hemodynamically stable, ambulating independently, and maintaining oxygen saturation above 95% on room air. The patient was tolerating a regular diet without difficulty and expressed understanding of and comfort with the discharge plan. Overall clinical trajectory was one of steady, uncomplicated improvement, and the patient was deemed medically stable and appropriate for discharge to home.

DISCHARGE MEDICATIONS AND CARE PLAN

The patient was instructed to complete the remaining prescribed course of oral antibiotics in full, even if feeling entirely well before the course is finished, to reduce the risk of incomplete treatment and recurrence. The existing antihypertensive medication regimen was continued unchanged, and the patient was advised to resume the as-needed bronchodilator inhaler only if experiencing wheeze or shortness of breath, consistent with prior use before this admission. No new long-term medications were added as a result of this hospitalization, and no medication dosing changes were made to existing home prescriptions.

DISCHARGE INSTRUCTIONS AND FOLLOW-UP

The patient was counseled extensively on the expected trajectory of recovery, including the likelihood of persistent mild cough and fatigue for one to two weeks following discharge, which should gradually continue to improve. The patient was advised to maintain adequate hydration, prioritize rest, and gradually increase activity as tolerated, avoiding strenuous exertion until follow-up evaluation. Clear return precautions were reviewed in detail, including instructions to seek immediate medical attention for recurrent or worsening fever, increasing shortness of breath, chest pain, confusion, or any new or concerning symptoms. Follow-up with the primary care physician was recommended within one week of discharge to reassess clinical status, with consideration of a follow-up chest radiograph in four to six weeks to confirm complete radiographic resolution, particularly given the patient's history of intermittent asthma. The patient verbalized understanding of all instructions provided, was able to correctly repeat back the key return precautions, and had no further questions at the time of discharge.

PATIENT EDUCATION PROVIDED

Prior to discharge, time was spent reviewing the nature of community-acquired pneumonia in plain language, including why completing the full antibiotic course matters even after symptoms improve, and why a lingering cough for several weeks after apparent recovery is common and not necessarily a sign that treatment has failed. The relationship between the patient's underlying mild asthma and respiratory infections was also discussed, with reinforcement of correct inhaler technique and a reminder to keep the rescue inhaler accessible during the recovery period. The patient was encouraged to space out physical exertion gradually over the coming two weeks rather than resuming prior activity levels abruptly, and was given written material summarizing the return precautions discussed verbally. The patient's questions regarding when it would be safe to resume normal activity were answered, with guidance to let symptom resolution and energy levels, rather than a fixed calendar date, guide the pace of that return.